Tarkastele tuloksia

Vastaaja		
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Survey objectives, target group and instructions

Objectives of this survey:
a. To begin to understand the expected added value of National Cancer Data Nodes (NCDNs)
b. To get a first reflection on the functional needs of NCDN (Section 2)
c. To understand the operational needs for Member States (Section 3)

Target group:
This survey is intended for representatives of Member States who are familiar with potential plans and stakeholders related to NCDNs in their respective countries.

Instructions:
Please respond to the survey to the best of your knowledge. You are welcome to consult relevant experts to inform your answers. However, there should be only one response per country.

The survey must be completed in a single session. A PDF copy of the survey is attached to this email for your reference. You may share it with colleagues you consult during the process.

You can set the interface language to English using the language selector in the top right corner.

Initial framework for National Cancer Data Nodes

Currently cancer research and care require a huge amount of complex and multimodal data often not readily available to a researcher or health care professional. Cancer data includes all cancer-related data (omics, imaging, clinical, registry data etc), generated from both health care and research, interpreted along diagnosis, treatment and outcome trajectories.

The aim for National Cancer Data Nodes (eCancer WG) is to facilitate the access of cancer digital assets from multiple sources by offering information and tools to researchers and national cancer data holders that complement EHDS infrastructure capabilities at national level and that leverage on the services already offered by current EU Research Infrastructures.

Furthermore, National Cancer Data Nodes aim to foster collaboration, capacity building and knowledge sharing in the field of cancer data among European cancer research institutions, national cancer research networks, data holders and data nodes, with the goal of accelerating research and innovation and improving patient outcomes.

Section 1: General Information

1. Respondent's name *

Zdenek Gütter

2. Respondent's email *

zdenek.gutter@mzcr.cz

3. Country of the respondent *

Czech Republic

4. In what role are you representing your country? *

Expert

5. Organization of the respondent *

Ministry of Health

6. Role in the organization *

Senior expert

7. Which stakeholder groups views are you familiar with? *

- [ ] Patients
- [ ] Clinicians
- [ ] Researchers
- [ ] Research infrastructures
- [x] Policy makers
- [ ] Muu

8. What is your role/mandate in your country's plans for setting up a National Cancer Data Node (NCDN)? *

External epxert in health data governance and allignment with the EHDS

9. Who else should be included as key players of NCDNs from your country in future developments?

Zdenka Dudova

Section 2. National Cancer Data Node Functions

The eCANCER Thematic Working Group identified the following five core functions of an NCDN:

1. Establish **national cancer data nodes** for cancer data holders
2. Improve **data quality** of cancer data at source
3. Ensure all cancer datasets have a **metadata record** in the EU dataset catalogue
4. Develop **common dataset variables** for cancer datasets
5. Provide **tools for cancer data analysis** to be used in secure processing environments (SPEs)

10. Please rank functions 2–5 by dragging them into an order, starting with the most relevant for your country's NCDN and ending with the least relevant. *

1 Ensure all cancer datasets have a **metadata record** in the EU dataset catalogue

2 Develop **common dataset variables** for cancer datasets

3 Improve **data quality** of cancer data at source

4 Provide **tools for cancer data analysis** to be used in secure processing environments (SPEs)

11. Are there additional functions you consider important for your country?

Cancer data standardization

2.1 Establish **National Cancer Data Nodes** for Cancer Data Holders

12. What types of cancer data exist in your country? *

- [x] Clinical data (data collected for healthcare purposes)
- [x] Research data (data collected for the purpose of scientific research)
- [x] Registry data
- [x] Biobank data
- [x] Imaging data
- [x] Genetic data
- [ ] Muu

13. Which types of organizations or data holders store cancer data in your country *

- [x] Hospitals/Clinics
- [x] Universities/Research Institutes
- [x] Registries
- [x] Biobanks
- [ ] Private Sector
- [ ] Muu

14. What types of data and in which format is stored by hospitals/clinics?

Answer if it applies to your country

Clinical data in diverse formats – national standard, partially OMOP and/or HL7 FHIR; imaging data in DICOM, TIFF...;

15. What types of data and in which format is stored by universities/research institutes?

Answer if it applies to your country

Mainly research onco data – usually in collaboration with hospitals/clinics

16. What types of data and in which format is stored by registries?

Answer if it applies to your country

The National Oncology Registr (NOR) was established in 1976, when all reported cases of neoplasms were monitored throughout their course. NOR data are also used to support the early diagnosis and treatment of neoplasms and precancerous conditions, to monitor trends in their occurrence, causal factors, and social consequences. At the population level, the results of neoplasm treatment are also evaluated in the form of survival analysis. The summary data serve as a basis for the creation, implementation, and evaluation of preventive health programs and for estimates of the financial costs necessary to provide comprehensive oncological care.
Standards: National standard for the format and data structure content; ICD 10, International Classification of Diseases for Oncology ICD-O-3, TNM Classification of Malignant Tumours.

Translated with DeepL.com (free version)

17. What types of data and in which format is stored by biobanks?

Answer if it applies to your country

Clinical data, Biospecimen data, Imaging data (CT,MRI,X-ray, WSI...), Genetic data (TSO, WGS, WES), Questionnaire data

18. What types of data and in which format is stored by private sector?

Answer if it applies to your country

N/A

19. What types of data and in which format is stored by other organizations or data holders?

Answer if it applies to your country

N/A

20. Are there already infrastructures or players in your country that support or connect cancer data holders, such as national nodes of European Research Infrastructures? *

- (●) Yes
- ( ) No
- ( ) I don't know

21. What in infrastructures or players supporting or connecting cancer data holders, such as national nodes of European Research Infrastructures, exist in your country? *

- [x] BBMRI-ERIC
- [ ] ELIXIR
- [x] EATRIS-ERIC
- [ ] Euro-BioImaging ERIC
- [x] ECRIN-ERIC
- [x] Federated EGA (FEGA)
- [ ] Muu

22. Are these infrastructures linked to other EU projects in your country?

Please name the projects in your answer.

Yes

23. Are there already infrastructures or players in your country that support or connect cancer data holders, such as national nodes of EU-funded data infrastructure initiatives? *

- (●) Yes
- ( ) No
- ( ) I don't know

24. What infrastructures or players exist in your country that support or connect cancer data holders, such as national nodes of EU-funded data infrastructure initiatives? *

- [x] GDI
- [x] EUCAIM
- [x] EOSC4Cancer
- [x] CanSERV
- [x] B1MGplus
- [x] TEHDAS2
- [x] CANDLE
- [ ] UNCAN-CONNECT
- [x] HealthData@EU Pilot
- [ ] Muu

25. Do the existing cancer data holders in your country collaborate? *

- (●) Yes
- ( ) No
- ( ) I don't know

26. How do the existing cancer data holders in your country collaborate? *

In example by sharing data, participating in a same project etc.

E.g. via National Institute for Cancer Research (Czech abbreviation NÚVR), Comprehensive Cancer Center of 1st Faculty of Medicine, Charles University, ad hoc platforms established during various research projects

27. Is there expertise in your country in data security to advise data holders on relevant issues within the EHDS? *

- [x] Yes
- [ ] No
- [ ] I don't know

28. Please describe the expertise there is in your country in data security to advise data holders on relevant issues within the EHDS. *

Providing expertise is not yet established systematically, organizationally. Partial expertize is developed especially with the frame of EOSC (Trusted Research Environment) and Czech membership in EU project EOSC-ENTRUST
Other stakeholders in this area are:
NCEZ/MoH CZ – the National eHealth Center tracks EHDS regulation milestones and national implementation obligations (creation of Digital Health Authority, HDAB, SPE security requirements, timelines)
ÚZIS ČR (Institute of Health Information and Statistics) – operates the National Health Information System (NZIS)
NÚKIB (National Cyber and Information Security Agency) – national guidance, minimum security standards, and sector advisories for healthcare (e.g., ransomware/DDoS readiness). New Act No. 264/2025 Sb. on Cybersecurity and implementing decrees entered into force in Nov 2025, expanding obligations on regulated services (including healthcare)
TEHDAS2 – EU-level practical guidance for Secure Processing Environments (SPEs) used under EHDS secondary use; Czech HDAB/SPE setup can draw directly on this guideline

29. What processes and mechanisms exist in your country to identify, interact with and/or coordinate national data holders? *

NZIS Open / NZIP – a single public portal with a catalogue of health data outputs (data if the National Health Information System), open data datasets, data summaries, interactive portals, plus a structured request workflow for one-off outputs and synthetic data access (prepare scripts on synthetic DB).
Data sharing concept by NZIS – sets access regimes, governance, and standard operating procedures (Advisory board NZIS; communication & data centre), and was officially approved by MoH on 6 May 2025.
Oncology-specific synthetic data (NOP Online) – modalities documented for NOR + NRHZS combinations; guidance for identification of oncology treatment in admin data.
BBMRI.cz/BBMRI-ERIC workflow - Researchers discover biobank collections and associated data through the BBMRI-ERIC Directory or Federated Platform, then submit access request via the Negotiator, which manages communication and agreements with all relevant data holders. For secondary use similar to the EHDS, the Federated Platform enables privacy-preserving queries within secured environments, ensuring compliance with DAAMS and Trusted Health Data Holder requirements without moving raw data.

30. Do you consider your country has the ability to participate in a European network of cancer data nodes? *

- [x] Yes
- [ ] No
- [ ] I don't know

2.2 Improve **Data Quality** of Cancer Data at Source

31. Are there any initiatives/projects in your country aiming to improve data quality at source for **clinical data**? *

- (●) Yes
- ( ) No
- ( ) I don't know

32. Please name the initiatives/projects aiming to improve data quality at source for **clinical data** in your country. *

Relevant EU project, Czech members in these projects are either academics or clinical staff.

33. Are there any initiatives/projects in your country aiming to improve data quality at source for **research data**? *

- (●) Yes
- ( ) No
- ( ) I don't know

34. Please name the initiatives/projects aiming to improve data quality at source for **research data** in your country. *

Relevant EU project, Czech members in these projects are academics. E.g. EOSC CZ initiative (https://www.eosc.cz/en/projects/national-support)

35. Are there any initiatives/projects in your country aiming to improve data quality at source for **registry data**? *

- (●) Yes
- ( ) No
- ( ) I don't know

36. Please name the initiatives/projects aiming to improve data quality at source for **registry data** in your country. *

UZIS national initiatives

37. Are there any initiatives/projects in your country aiming to improve data quality at source for **biobank data**? *

- (●) Yes
- ( ) No
- ( ) I don't know

38. Please name the initiatives/projects aiming to improve data quality at source for **biobank data** in your country. *

BBMRI.cz started with Federated Data Quality Framework (Data Quality Framework) PILOT on HL7 FHIR.

39. Are there any initiatives/projects in your country aiming to improve data quality at source for **imaging data**? *

- (●) Yes
- ( ) No
- ( ) I don't know

40. Please name the initiatives/projects aiming to improve data quality at source for **imaging data** in your country. *

BioImaging (https://www.czech-bioimaging.cz/) or Neuroscientists at CEITEC (https://mafil.ceitec.cz/en/)

41. Are there any initiatives/projects in your country aiming to improve data quality at source for **genetic data**? *

- (●) Yes
- ( ) No
- ( ) I don't know

42. Please name the initiatives/projects aiming to improve data quality at source for **genetic data** in your country. *

EU projects following the 1+million Genome initiative, such as B1MGplus with Czech members (the national initiatives are coordinated by MUNI Brno)

43. What functions are required to improve cancer data quality at source in your country? *

Clear standard (at least compilation of existing standards necessary for onco data – no new standard is expected), harmonisation according to the standard, technical knowledge (e.g. HL7 FHIR and OMOP experts)

44. What support or guidance would you need to ensure high-quality, FAIR-compliant data from the outset? *

Planning and commitment from all stakeholders, coordination

45. What are the bottlenecks and opportunities? *

Bottlenecs: systematic uptake of research findings in healthcare
Opportunities: EU wide sharing of knowledge

2.3 Ensure All Cancer Datasets Have a **Metadata Record** in the EU Dataset Catalogue

46. In your country, is there any initiative defining a national metadata record of all cancer data sets? *

- ○ Yes
- ○ No
- ◉ I don't know

47. Please name the initative(s) defining a national metadata record of all cancer data sets in your county. *

Czechia does not have a single, comprehensive national metadata registry of all cancer datasets across clinical, administrative, research and biobank spheres. Significant partial catalogues exist (see below), but they do not exhaustively list every cancer dataset nationally
NZIP – Data reporting / Health data catalog (incl. NOR outputs, oncology open data/summaries) [nzip.cz]
SVOD – Portal of Cancer Epidemiology (metadata around incidence, mortality, prevalence, survival). [svod.cz]
NKOD (National Catalogue of Open Data) – national DCAT-AP-CZ metadata hub; ÚZIS publishes health open datasets (e.g., NRPZS). [nrpzs.uzis.cz], [data.gov.cz]
BBMRI-ERIC Directory/Negotiator (via BBMRI-CZ) – pan-European biobank catalogue with Czech node collections (samples + associated clinical data). [bbmri-eric.eu], [bbmri.cz]
CZECRIN ONCO / GENESIS (MTB outcomes) – disease-oriented network and registry elements for academic oncology (metadata on MTB results).

48. What standard is used in the initiative(s) defining a national metadata record. Please select all that apply. *

- [x] CDISC
- [x] The 1 +MG Minimal Dataset for Cancer
- [x] GDI Minimal Cancer Dataset
- [ ] DigiOne MEDOC
- [x] DCAT-AP
- [ ] I don't know
- [ ] Muu

49. What is the status of the initiatives?

NZIP/NZIS Open concept formally approved (May 2025), with live catalogue and synthetic data modality. [nzip.cz]
NKOD/DCAT-AP-CZ – updated profiles (2024–2025) incl. DCAT-AP 3.0.0/3.0.1 and HVD support. [data.gov.cz], [ofn.gov.cz]
SVOD – long-running portal continuously maintained. [svod.cz]
BBMRI-CZ/Directory/Negotiator – operating; Czech biobanks listed; Negotiator open-source maintained by BBMRI-ERIC.
GDI/1+MG MDC – active EU project; MDC published & being operationalised in cancer use-cases.

50. What are the bottlenecks and opportunities? *

Bottlenecks: fragmented holders; lack of a single national catalogue spanning clinical+research+biobank; mixed standards across domains.
Opportunities: DCAT-AP-CZ evolution in NKOD; NZIS Open governance; SPE-based secondary use (SensitiveCloud secure environment of e-INFRA CZ); genomics MDC to harmonise cancer metadata across nodes.

2.4.1 Develop **Common Dataset Variables** for Cancer Datasets - **Structural Standardization**

Definition of structural standardization: The organization of data elements and the definition of their relationships within a data model.

51. Are there any national initiatives in your country that define common dataset variables for cancer data on a structural level (e.g. national cancer registry schemas covering patient demographics, tumor characteristics, treatment, outcomes)? *

- (●) Yes
- ( ) No
- ( ) I don't know

52. Please describe these national structural standardization initiative(s) and their status. *

NOR registry schema – variables for diagnosis, morphology (ICD-O-3), TNM/staging, demographics, treatment; ENCR/IACR alignment. [uzis.cz]
Linked NZIS components – NRHZS (claims-based longitudinal treatment), LPZ (mortality) complement NOR analytics; reflected in SVOD/NZIP outputs.

NOR continuously updated; post-2019 electronic data capture methodologies refreshed; NZIS synthetic oncology datasets published for expert analysis workflows

53. Is your country part of any of the following or other EU initiatives defining structural data standardization? *

- [x] HL7 Europe / HL7 FHIR
- [x] GA4GH
- [ ] DarwinEU
- [ ] EHDEN
- [x] OpenEHR
- [ ] No
- [ ] Muu

54. What is the status of these structural standardization EU initiatives in your country? *

HL7 FHIR – Czech Base/Core IG in ballot (R4), national adoption path defined by NCEZ for MyHealth@EU domains. [build.fhir.org], [ncez.mzcr.cz]
EHDEN/OMOP – data partners onboarded and mapping to OMOP CDM; network studies feasible. [ehden.eu]
GDI/1+MG MDC – ongoing; Czech institutions (Masaryk University, CEITEC/CERIT-SC) are part of project governance/infra pillars. [1mg.cz]

55. Which institutions and/or organizations are involved within the country in the initiatives?

MoH (NCEZ), ÚZIS ČR, Masaryk University (CEITEC, CERIT-SC; GDI; SVOD), BBMRI-CZ (MMCI), Palacký University (IMTM, EATRIS-CZ), CZECRIN (MU+FNUSA), Institute of Rheumatology & Czech Myeloma Group & Jihomoravský kraj (EHDEN)

56. Which standards or data models are currently used or under consideration to standardize cancer data on a structural level in your country? *

- [x] HL7 FHIR
- [x] OMOP CDM
- [ ] OpenEHR
- [ ] None
- [ ] Muu

57. In which areas do you anticipate that existing standards are insufficiently specific for cancer data, and what gaps should be addressed?

Longitudinal oncology pathways – capturing lines of therapy, response criteria, relapse/progression, toxicities and real-world sequencing remains uneven across EHR/FHIR and claims; consistent, pan-EU FHIR oncology profiles are still maturing.
Molecular pathology and biomarkers – granular variant reporting, panels, tumour mutational burden, MSI status, clonal evolution require oncology-specific profiles and harmonised terminologies across FHIR/OMOP.
Crosswalks registry ↔ EHR/OMOP – mapping ICD-O-3/TNM and NOR variables to FHIR/OMOP consistently for federated analytics needs national guidance and public code-lists.
SPE/HDAB operational specifics – oncology cohorts often involve sensitive linkages; practical export policies (aggregations, k-anonymity) under SPEs must be tailored.

58. What are the main bottlenecks and opportunities in implementing structural data standards for cancer data? *

Fragmentation of sources & legacy formats (e.g., national Data standard for data transfer
between healthcare facility information systems (DASTA) vs emerging FHIR) and heterogeneity between clinical registries and hospital EHRs.
Compliance workload – new cybersecurity law (NIS2 transposition) increases organisational obligations; smaller providers may struggle with capacity.
Data access risk management – oncology data are highly re-identifiable; robust anonymisation and synthetic-data-driven workflows are essential but resource-intensive.

2.4.1 Develop **Common Dataset Variables** for Cancer Datasets - **Semantic Standardization**

Definition of semantic standardization: The adoption of a common (standardized) nomenclature that enables consistent interpretation of data across systems.

59. Are there national initiatives in your country that define common dataset variables for cancer data at the semantic level? *

- ( ) Yes
- ( ) No
- (•) I don't know

60. Is your country involved in the following or other EU or international initiatives for semantic data standardization? *

- [ ] UNCAN.eu
- [x] EUCAIM
- [ ] IDEA4RC
- [x] TEHDAS2
- [ ] None
- [ ] Muu

61. Which semantic standards, ontologies or terminologies are currently used to standardize data on a semantic level in your country? *

- [x] SNOMED CT
- [ ] ICD-O
- [x] LOINC
- [ ] None
- [ ] Muu

62. Which semantic standards, ontologies or terminologies would you consider adopting for specific data types (clinical data, imaging data, genomics data, registry data etc)? *

ICD 11, ICD-O-3, SNOMED CT, LOINC

63. What are the main bottlenecks and opportunities in implementing semantic data standardizations for cancer data? *

N/A

2.5 Provide **Tools for Cancer Data Analysis** to be Used in Secure Processing Environments*

*Secure processing environments (SPEs)

64. In your country, is there any initiative defining SPEs for cancer data analysis? *

- ○ Yes
- ○ No
- ◉ I don't know

65. Are there defined software tools in the SPEs in cancer research used in your country?

- ○ Yes
- ○ No
- ◉ I don't know

66. Are there established standards or requirements for the quality of the software tools? *

- ○ Yes
- ○ No
- ◉ I don't know

67. Is your country actively participating in EU initiatives preparing for EHDS (including Digital Business Capabilities, like analytics for Secure Processing Environments) such as TEHDAS2, Community of Practice for Secondary Use of Health Data etc? *

- [x] Yes
- [ ] No
- [ ] I don't know

68. Please describe the initiative(s) and their status. *

EHDS2 Community of Practice, TEHDAS2

69. What standards are used in these initiative(s)? *

See TEHDAS2 deliverable M7.4 Draft technical, functional and security specifications of Secure Processing Environments (in update process in 12.2025), includes ISO/IEC 27000 family of standards

EVERSE RSQKit, Software Quality, "IEEE Standard 730-2014

70. Are there local EHDS implementation projects in your country? *

- [x] Yes
- [ ] No
- [ ] I don't know

71. Are specific Secure Processing Environments (SPEs) intended to be used there? *

- [ ] Yes
- [x] No
- [ ] I don't know

72. What are the bottlenecks and opportunities? *

Opportunities: collaboration wijh EOSC (with TRE)

73. What kind of infrastructures, tools and platforms would be most useful to support cancer data analysis and sharing in your country? *

N/A

74. What mechanisms are in place to ensure secure data access and present re-identification?

N/A

2.6 Possible Additional Function: Collecting Data from **Diverse Sources**

One additional function of an NCDN could be collecting data either physically or virtually on a national level from diverse sources such as cancer registries, hospital information systems, pathology and genomics labs, imaging archives (e.g. PACS) and clinical trials and research cohorts.

75. Is collecting data from diverse sources an important possible additional function for a NCDN in your opinion? *

- (●) Yes
- ( ) No

76. Please explain the reasoning behind your response.

Facilitating the search for and access to relevant datasets

2.7 Possible Additional Function: Community Engagement

77. In your Member state, is there already an initiative bringing national cancer data related activities together? *

- ( ) Yes
- (●) No
- ( ) I don't know

78. Can you name people or organizations in your country that you consider would be important in developing the NCDN functions at a later stage in your country?

Participants in CANDLE (national working group)

Section 3: NCDN Operations

Questions of this section start on next page

3.1 General Stage of NCDN Development

79. Does your country have a national cancer data strategy? *

- (●) Yes
- ( ) No
- ( ) I don't know

80. How is the strategy like? *

Initial considerations and proposals how to align the node with the EHDS are made

81. In what stage is the preparation of an NCDN in your country? *

initial stage

82. What are your key considerations for the establishment of an NCDN? *

Opting for Trusted data holders (persuant to the EHDS Regulation) to be established at the major cancer data holders. These trusted data holders may be created for all categories of health data that the particular institution (hospital) holds. Data holders will keep the oncology data in line with their policies, using all the same standards and data govenance, aligned with the EHDS Regulation. No replication, transfer of the data to a centre outside of contol of the data holders is planned. NCDN will have coordination and support role for all oncology data and their data holders, be they trusted or ordinary data holders. It will provide also guidance on quality of data and for tools for cancer data analysis. NCDN will be the contact point for EU /international coordination and collaboration in oncology data. NCDN will have knowledge about all data holders that enable access to oncology data the Czech Rep. NCDN should not create parallel platform for access to the data; the oncology data will be part of EHDS, relevant mechanisms defined by the EHDS Reg. will be used. A cancer data catalogue role is initially outlined and it would be to make it easier to search for oncology data and possibly provide additional information about the datasets and capabilites of the relevant data holders regarding specific processing of the data (such as linkage, creation of subsets, optionally harmonization of the data, analytic processing). Oncology data catalogue may be operated by the NCDN. This oncology data catalogue is considered to be supplementary to the EHDS national health data catalogue, which should contain all metadata of all national health data as required by the EHDS Regulation, and this national catalogue will be used for selection of dataset for the data request submitted by the data user. The rationale behind this concept is that users of oncology data may have specific search requirements on the available datasets that are normally not supported by HealthDCAT-AP standard, but the oncology data catalogue may support them. This concept of data catalogues will require to define the specific properties of the oncology datasets above the existing properties in HealthDCAT-AP and additional information, then to develop/amend suitable software solution and define guidelines for it, and also define the method of technical interworking with national health data catalogue. It is recommended to perform a feasibility study for this.

83. To your knowledge, is your country currently developing or planning to develop an NCDN in alignment with the EHDS development? *

- (●) Yes
- ( ) No
- ( ) I don't know

84. How will the national laws (e.g. EHDS regulations) influence NCDN operation?

EHDS Reg. in many ways, to a large extent.

3.2 Key players in NCDN governance

85. Please list key players in governance and operations of a potential NCDN in your country (e.g. ministries, HDABs, RIs, cancer data holders). *

MoH, HDAB (which is to be nominated), 3 major oncology centres - data holders, national oncology register in UZIS, oncology reseach community, other oncology data holders, other RI

86. Are the key players the same or associated with institutions who are responsible for EHDS implementation in your country? *

- ( ) Yes
- (●) No
- ( ) I don't know

87. In case you already know, please describe how will the decisions be made about data access, prioritization, or oversight in your country's NCDN?

As outined above, national infrastructure for compliance with the EHDS is planned to be used. This includes: Trusted data holders, HDAB with its national data catalogue and DAAMS, ordinary data holders, SPE, NCP2.

88. Who do you think is the most appropriate person or organization to answer the questions in the NCDN Operations section, and are there others you believe should also be involved?
Please list all relevant individuals or organizations.

Czech members in the project, incl. Zdenka Dudova external collaboration - Zdenek Gütter, MoH, others may be added later.